Order No:485-25314

## HOME HEALTH CERTIFICATION AND PLAN OF CARE

| 1. Patient's HI Claim No. | 2. Start Of Care Date | 3. Certification Period | 4. Medical Record No. | 5. Provider No. |
|---|---|---|---|---|
| 9FC2W27QR92 | 12/10/2025 | 02/08/2026 - 04/08/2026 | 6132 | 747688 |

**6. Patient's Name and Address**
Reyna, Roy (6132)
607 Filmore Ave. #704
Victoria, TX 77901
Phone: (361) 759-0890

**7. Provider's Name, Address and Telephone Number**
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

**8. Date of Birth:** 10/30/1959 | **9. Sex:** ☒ M ☐ F

**11. ICD-9 / 10 CM** **Principal Diagnosis**

| Code | Code Description | Date | O/E |
|---|---|---|---|
| I11.0 | Hypertensive heart disease with heart failure | | |

**12. ICD-9 / 10 CM** **Surgical Procedure**

| Code | Code Description | Date | O/E |
|---|---|---|---|
| | | | |

**13. ICD-9 / 10 CM** **Other Pertinent Diagnosis**

| Code | Code Description | Date | O/E |
|---|---|---|---|
| I50.33 | Acute on chronic diastolic (congestive) heart failure | | |
| E11.42 | Type 2 diabetes mellitus with diabetic polyneuropathy | | |
| I48.0 | Paroxysmal atrial fibrillation | | |
| K70.31 | Alcoholic cirrhosis of liver with ascites | | |
| E22.2 | Syndrome of inappropriate secretion of antidiuretic hormone | | |
| E83.42 | Hypomagnesemia | | |
| D64.9 | Anemia, unspecified | | |
| E78.5 | Hyperlipidemia, unspecified | | |
| Z91.81 | History of falling | | |
| Z86.59 | Personal history of other mental and behavioral disorders | | |
| Z79.01 | Long term (current) use of anticoagulants | | |

**14. DME and Supplies:**
DME: Walker
ALCOHOL WIPES, PROBE COVERS, NS GLOVES, PPE MASK, DM SUPPLIES, SHARPS CONTAINER

**15. Safety Measures:**
911 Emergency / Protocol, 911 for Emergency, Anticoagulant Precautions, Assistance with IADLs, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Keep Pathways Clear and Safe, Medication Safety / Storage, Safe Disposal of Sharp, Contaminated Materials, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

**16. Nutritional Req.**
Sodium Diet: Low, No Concentrated Sweets,

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**8. Date of Birth:** 10/30/1938 | **9. Sex:** ☒ M ☐ F

HEART HEALTHY/CHF

**17. Allergies:**
No Known Allergies

**18.A. Functional Limitations**
☐ 1- Amputation ☒ 4- Hearing ☒ 7- Ambulation
☐ 2- Bowel/Bladder (Incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☒ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

**18.B. Activities Permitted**
☐ 1- Complete Bedrest ☒ 5- Exercises Prescribed ☒ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☐ 7- Independent at Home ☒ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☒ D- Other (Specify):SBA

**19. Mental Status**
☒ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

**20. Prognosis:** ☐ 1- Poor ☐ 2- Guarded ☒ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

**21. Treatments and Orders for Disciplines:**
**Frequency**
SN Frequency: 1W5, 1EOW4 Starting Week of 02/08/2026

**Statement**

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify in my estimation that the patient will need skilled care from home health for the next 60 days.
I attest to the following:
1. Roy Reyna is under my care.
2. I have received the following patient records from Be Healthy at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
3. Each of these patient records have been incorporated into my patient record.

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☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☒ D- Other (Specify):SBA

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**8. Date of Birth:** 10/30/1938 | **9. Sex:** ☒ M ☐ F

4. These patients records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

**Vitals Normal Range**

BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

**Vitals Physician Alert Range**

Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 250
Random Blood Sugar: < 70 or > 300
Weight: < 159 or > 169
Pain: > 7

**SN**

**ANTICOAGULANT THERAPY:**

- Clinician will assess/instruct patient/caregiver on anticoagulant therapy, to include diet, safety, reportable s/s.
- SN to instruct patient/caregiver on medication side effects to report to SN or Physician.
- SN to instruct the patient/caregiver on precautions for high risk medications, such as, hypoglycemics, anticoagulants/antiplatelets, sedative hyponotics, antiarrhythmics, antineoplastics, and skeletal muscle relaxants.

**BEST PRACTICE FALL PREVENTION:**

- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

**BEST PRACTICE MEDICATIONS/DRUG EDUCATION:**

- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

**BEST PRACTICE MONITOR AND MITIGATE PAIN:**

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

**BEST PRACTICE PRESSURE ULCER PREVENTION:**

- Clinician will perform validated test Braden to assess patient for risk of development of Pressure ulcers.
- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

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8. Date of Birth: 10/30/1958 | 9. Sex: ☒ M ☐ F

**CARDIOVASCULAR: CHF:**

- Clinician to assess/instruct patient/caregiver Symptoms in Heart failure indicated by clinical heart failure guidelines (including dyspnea, orthopnea, edema, or weight gain).

**CHF:**

- SN to instruct on edema management

**DIABETES:**

- SN to instruct patient and family on signs of hyperglycemia to include: Polyphagia- being hungry frequently, pronounced hunger Polydipsia- beingfrequently thirsty, excessive thirst; Polyuria- frequently urinating; Headaches; Difficulty in concentrating; Blurred vision; Weak and tired feeling;Weight loss; Blood glucose level is more than 180 mg/dl.
- SN to instruct patient and family on signs and symptoms of hypoglycemia to include: Confusion, Dizziness, Feeling shaky, Hunger, Headaches,Irritability, Pounding heart; racing pulse. Pale skin Sweating Trembling Weakness Anxiety

**GENITOURINARY:**

- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.
- Clinician to assess patient for presence of bowel and bladder incontinence. Instruct patient /caregiver measures to improve incontinence.

Instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.

**HOSPITALIZATION RISK FACTORS:**

- Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: HYPERTENSIVE HEART DISEASE WITH HEART FAILURE. DM2, HTN, HLD, AFIB
- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

**CARDIOVASCULAR - HTN:**

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

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**CARDIOVASCULAR - AFIB:**

- SN to instruct pt/cg on s/s of Afib such as: weakness, rapid or irregular heartbeat, fluttering or thumping in the chest, dizziness, SOB, or lightheadedness
- SN to instruct pt/cg on how to manage Afib such as: eating a heart healthy diet, take medication as prescribed, avoid common triggers (infections, heart failure, alcohol, caffeine, skipping medications, smoking), maintaining a healthy BMI, and exercising as recommended by the doctor.
- SN to instruct pt/cg on s/s stroke and when to notify EMS
- SN to instruct patient/caregiver on high-risk medications anticoagulants, and antiplatelet medications as prescribed.

**60 Day Recert Summary**

Physician: NAILA KHAN MD
Date: 02/03/2026
Vital Sign Range:
Temperature: 97.3 - 98.6
Pulse: 79 - 112
Respiration: 18 - 18
Blood Pressure
Systolic: 112 - 134
Diastolic: 77 - 90
O2 Saturation: 93 - 100
Labs: n/a
Care Summary: RECERT
FOCUS OF CARE: HYPERTENSIVE HEART DISEASE WITH HEART FAILURE, DM2, HLD, AFIB
RECERT of a 66 year old male with a past medical hx of DM2, HTN, HLD, HEART FAILURE, AFIB. He went to CMC ER 11/30/25 with a 3 day hx of gradually increasing swelling of hands, feet, and legs as well as moderate dyspnea on exertion. CBC showed Anemia, pt was admitted with nephrology consult and placed on IV Lasix for diuresis. He required ongoing multi-disciplinary therapy under physician supervision and was admitted to PAM IPR on 12/2/25, discharged home on 12/9/25. Patient needs skilled nursing for teaching and monitoring related to heart failure, low sodium and magnesium causing weakness and multiple falls, diabetes, and fall prevention. Patient is weak, has trouble walking, and needs help with medications and staying safe at home.
The patient resides in a first-floor apartment with his life partner, who is able and willing to assist with care. His adult son lives nearby and is available to assist as needed. The patient ambulates using a front-wheeled walker and requires standby assist for mobility and ADLs. He reports frequent falls prior to admission, and currently utilizes DME including walker, grab bars, and shower chair.
The patient is homebound due to generalized weakness, unsteady gait, impaired balance, and the need for assistance with mobility and personal care, making it a taxing effort to leave the home without support.
The patient is being recertified to PAM HH for skilled nursing assessments, chronic disease management, medication education, fall prevention, and safety monitoring. No wounds were present at this time. SN to monitor skin integrity and educate the patient on hygiene, pressure relief, and safety practices. Medication reconciliation was completed. Education provided on 1200 mL fluid restriction and the importance of performing daily weights for heart failure management -pt does check weight daily. Ongoing teaching will be required to support safe recovery at home.
Pt saw PCP this past week and has an appointment today at 2pm with Dr. Kumar
Educated today on Educated patient on CHF symptom zones:

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Green Zone (Stable) – No shortness of breath, no weight gain, no swelling, able to perform usual activities.
Yellow Zone (Caution) – Weight gain of 2–3 lbs in 1–2 days, increased swelling in feet/legs, mild shortness of breath with activity, dry cough, or feeling more tired than usual; instructed to notify home health nurse or physician.
Red Zone (Danger) – Weight gain of more than 5 lbs in 1 week, shortness of breath at rest, chest pain, confusion, or needing to sleep upright due to breathing difficulty; instructed to seek immediate medical attention or call 911.
The patient verbalized understanding of the current plan and stated his goals for home health are to feel safe at home, gain strength, prevent falls, and learn to manage his health conditions more independently.

**Risk of Hospitalization**

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;
Interventions: Referrals - SN; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...); Tele monitoring
Risk Factors: Help with managing Medications needed (M2020); Confusion (M1710); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

**Psychosocial Assessment**

No problem found

**Advance Directive**

Patient/Caregiver reports no Advanced Directives in place

**Homebound Status**

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Severe SOB, SOB upon exertion; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

**22. Rehabilitation Potential/Discharge Plans/Goals :**

**Recertification of Patient Eligibility**

I estimate the duration of continued Home Health services for this patient to be
60 days

**Rehab Potential**

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care

**Discharge Plan**

Discharge when medical condition is stable and patient is no longer in need of skilled services

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## HOME HEALTH CERTIFICATION AND PLAN OF CARE

| 1. Patient's HI Claim No. | 2. Start Of Care Date | 3. Certification Period | 4. Medical Record No. | 5. Provider No. |
|---|---|---|---|---|
| 9FC2W27QR92 | 12/10/2025 | 02/08/2026 - 04/08/2026 | 6132 | 747688 |

**6. Patient's Name and Address**
Reyna, Roy (6132)
607 Filmore Ave, #704
Victoria, TX 77901
Phone: (361) 759-0890

**7. Provider's Name, Address and Telephone Number**
PAM Health at Home
826 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

**8. Date of Birth:** 10/30/1958 | **9. Sex:** ☒ M ☐ F

**CARDIOVASCULAR: CHF:**

- Clinician to assess/instruct patient/caregiver Symptoms in Heart failure indicated by clinical heart failure guidelines (including dyspnea, orthopnea, edema, or weight gain).

**CHF:**

- SN to instruct on edema management

**DIABETES:**

- SN to instruct patient and family on signs of hyperglycemia to include: Polyphagia- being hungry frequently, pronounced hunger Polydipsia- beingfrequently thirsty, excessive thirst; Polyuria- frequently urinating; Headaches; Difficulty in concentrating; Blurred vision; Weak and tired feeling;Weight loss; Blood glucose level is more than 180 mg/dl.
- SN to instruct patient and family on signs and symptoms of hypoglycemia to include: Confusion, Dizziness, Feeling shaky, Hunger, Headaches,Irritability, Pounding heart; racing pulse. Pale skin Sweating Trembling Weakness Anxiety

**GENITOURINARY:**

- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.
- Clinician to assess patient for presence of bowel and bladder incontinence. Instruct patient /caregiver measures to improve incontinence.

Instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.

**HOSPITALIZATION RISK FACTORS:**

- Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: HYPERTENSIVE HEART DISEASE WITH HEART FAILURE. DM2, HTN, HLD, AFIB
- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

**CARDIOVASCULAR - HTN:**

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

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**CARDIOVASCULAR - AFIB:**

- SN to instruct pt/cg on s/s of Afib such as: weakness, rapid or irregular heartbeat, fluttering or thumping in the chest, dizziness, SOB, or lightheadedness
- SN to instruct pt/cg on how to manage Afib such as: eating a heart healthy diet, take medication as prescribed, avoid common triggers (infections, heart failure, alcohol, caffeine, skipping medications, smoking), maintaining a healthy BMI, and exercising as recommended by the doctor.
- SN to instruct pt/cg on s/s stroke and when to notify EMS
- SN to instruct patient/caregiver on high-risk medications anticoagulants, and antiplatelet medications as prescribed.

**60 Day Recert Summary**

Physician: NAILA KHAN MD
Date: 02/03/2026
Vital Sign Range:
Temperature: 97.3 - 98.6
Pulse: 79 - 112
Respiration: 18 - 18
Blood Pressure
Systolic: 112 - 134
Diastolic: 77 - 90
O2 Saturation: 93 - 100
Labs: n/a
Care Summary: RECERT
FOCUS OF CARE: HYPERTENSIVE HEART DISEASE WITH HEART FAILURE, DM2, HLD, AFIB
RECERT of a 66 year old male with a past medical hx of DM2, HTN, HLD, HEART FAILURE, AFIB. He went to CMC ER 11/30/25 with a 3 day hx of gradually increasing swelling of hands, feet, and legs as well as moderate dyspnea on exertion. CBC showed Anemia, pt was admitted with nephrology consult and placed on IV Lasix for diuresis. He required ongoing multi-disciplinary therapy under physician supervision and was admitted to PAM IPR on 12/2/25, discharged home on 12/9/25. Patient needs skilled nursing for teaching and monitoring related to heart failure, low sodium and magnesium causing weakness and multiple falls, diabetes, and fall prevention. Patient is weak, has trouble walking, and needs help with medications and staying safe at home.
The patient resides in a first-floor apartment with his life partner, who is able and willing to assist with care. His adult son lives nearby and is available to assist as needed. The patient ambulates using a front-wheeled walker and requires standby assist for mobility and ADLs. He reports frequent falls prior to admission, and currently utilizes DME including walker, grab bars, and shower chair.
The patient is homebound due to generalized weakness, unsteady gait, impaired balance, and the need for assistance with mobility and personal care, making it a taxing effort to leave the home without support.
The patient is being recertified to PAM HH for skilled nursing assessments, chronic disease management, medication education, fall prevention, and safety monitoring. No wounds were present at this time. SN to monitor skin integrity and educate the patient on hygiene, pressure relief, and safety practices. Medication reconciliation was completed. Education provided on 1200 mL fluid restriction and the importance of performing daily weights for heart failure management -pt does check weight daily. Ongoing teaching will be required to support safe recovery at home.
Pt saw PCP this past week and has an appointment today at 2pm with Dr. Kumar
Educated today on Educated patient on CHF symptom zones:

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Green Zone (Stable) – No shortness of breath, no weight gain, no swelling, able to perform usual activities.
Yellow Zone (Caution) – Weight gain of 2–3 lbs in 1–2 days, increased swelling in feet/legs, mild shortness of breath with activity, dry cough, or feeling more tired than usual; instructed to notify home health nurse or physician.
Red Zone (Danger) – Weight gain of more than 5 lbs in 1 week, shortness of breath at rest, chest pain, confusion, or needing to sleep upright due to breathing difficulty; instructed to seek immediate medical attention or call 911.
The patient verbalized understanding of the current plan and stated his goals for home health are to feel safe at home, gain strength, prevent falls, and learn to manage his health conditions more independently.

**Risk of Hospitalization**

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;
Interventions: Referrals - SN; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...); Tele monitoring
Risk Factors: Help with managing Medications needed (M2020); Confusion (M1710); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

**Psychosocial Assessment**

No problem found

**Advance Directive**

Patient/Caregiver reports no Advanced Directives in place

**Homebound Status**

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Severe SOB, SOB upon exertion; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

**22. Rehabilitation Potential/Discharge Plans/Goals :**

**Recertification of Patient Eligibility**

I estimate the duration of continued Home Health services for this patient to be
60 days

**Rehab Potential**

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care

**Discharge Plan**

Discharge when medical condition is stable and patient is no longer in need of skilled services

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**Goals**

**SN**

**ANTICOAGULANT THERAPY: - LTG**
- LTG-Patient will be free of adverse s/s of anticoagulant therapy throughout episode.

**BEST PRACTICE FALL PREVENTION: - LTG**
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

**BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG**
- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

**BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG**
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

**BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG**
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

**CARDIOVASCULAR: CHF: - LTG**
- LTG-Patient/caregiver will receive education on Symptoms indicative of heart failure and will be able to recall S/s of Heart failure and when to report by end of episode.

**DIABETES: - LTG**
- Hypo/Hyperglycemia sign and symptom free by end of episode.

**GENITOURINARY: - LTG**
- LTG-Patient will tolerate collection of urine without pain and discomfort throughout episode.

**HOSPITALIZATION RISK FACTORS: - LTG**
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

**CARDIOVASCULAR - HTN: - LTG**
- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

**CARDIOVASCULAR - AFIB: - LTG**
- Patient/caregiver will be able to verbalize s/s of Atrial fibrillation, and strategies to manage Afib.

**23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)**
02/03/2026
Signed By: Saunders, Shannon - RN on
02/03/2026 11:04 AM, Cosigned by
Strait, Donna - RN

**25. Date HHA Received Signed POT:**

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**24. Physician's Name and Address**
Julia Flores Mitchell MD
2700 Citizens Plaza STE 403 Bld A
VICTORIA, TX 77901,NPI: 1699160440, License#: R6050,Phone: (361) 582-8685, Fax: (361) 582-5613

**26.** I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

**27. Attending Physician's Signature and Date Signed:** 03/03/26

**28.** Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.

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